

Discharge Summary USE BLACK INK ONLY

Patient Details		Admission and GP Details	
Surname Whitfield		Discharging Consultant Dr F. Osei	
Forename Harold	M / F/ (Male).....	Discharging Speciality/ Department Resp Med / COTE	
Date of Birth 02/11/1943		Method of Admission Emergency - ambulance (home)	
NHS/ Hosp No. 872 419 3305		Date of Admission 16/07/2026	
Address 8 Orchard Grove,		Date of Discharge 25/07/2026	
Basingstoke, RG21 5FJ		G.P. Details Dr N. Patel,	
Tel No. 01256 778 3345 / NOK 07712 345678		Kingsclere Road Surgery, Basingstoke	
Diagnosis at Discharge		Operations and Procedures	
Community-acquired pneumonia (right lower lobe); infective exacerbation of COPD. PMH: T2DM, hypertension, CKD stage 3a, permanent AF (on apixaban).		None.	
Reason for Admission and Presenting Complaint(s)			
3-day history of worsening productive cough with green sputum, fevers and increasing breathlessness; daughter noted new confusion. Found hypoxic (SpO2 88% on air) with reduced air entry at right base.			
Clinical Narrative			
82M with COPD, T2DM, hypertension, CKD3a and AF presented with sepsis secondary to CAP with acute delirium. CXR confirmed right lower lobe consolidation. Treated with IV co-amoxiclav and clarithromycin, then stepped down to oral, with nebulised bronchodilators and controlled oxygen therapy (target 88-92% given COPD). Delirium resolved fully with treatment of sepsis, hydration and reorientation. Blood glucose monitored and diabetic medications adjusted during acute illness. Renal function remained at baseline CKD3a throughout. Physiotherapy input given. Discharged back to baseline mobility with frame, sats 94% on air.			
Relevant Investigations and Results			
WCC 16.8, CRP 210 falling to 34. Cr 142 (baseline), eGFR 38. CXR: RLL consolidation. Sputum: H. influenzae.			
Discharge Destination			
Home with existing package of care (2x daily); OT/PT assessed, safe for discharge with frame.			
Relevant legal Information (e.g. was an independent Mental Capacity Act Advocate required)			
Transient delirium during admission, now fully resolved. Capacity assessed and intact at discharge; no ongoing MCA concerns.			
Information given to patient and/or authorised representative (including e.g. see GP in 2 weeks)			
Diagnosis and recovery plan discussed with patient and daughter (with consent). Both counselled on signs of worsening chest infection, completing antibiotics and correct inhaler use. Advised to seek urgent help if breathless, feverish or confused again.			
Physical Ability & Cognitive Function :		On Admission	At Discharge
Physical	Needs standby help	Back to baseline, mobilises independently with frame	
Cognitive	Acute delirium	Resolved; baseline mild memory impairment only	
Other	Hypoxic, 2L O2	SpO2 94% on room air	
Advice, recommendations and future plans (including results awaited and outstanding investigations)			
G.P. Actions (Date) Recheck U&E and CRP in 1 week; review inhaler technique at next review.			
For GP: recheck U&E/CRP in 1 week (AKI on CKD3a); Ramipril withheld - review and restart once renal function stable; review COPD management, consider pulmonary rehab referral; check flu/pneumococcal vaccination status.			
Patient/daughter counselled as above re infection signs, antibiotic compliance and falls prevention; referral made to community falls clinic.			
Strategies for potential problems			
Daughter aware/involved in care; has contact details for community matron and falls team.			

